

Apollo Multispecialty Hospital

154/11, Bannerghatta Road, Opposite IIM, Bengaluru, Karnataka 560076
Ph: +91-80-2630-4050 | NABH Accredited | Reg. No: KA/BLR/MH-2019/04872

DISCHARGE SUMMARY & CLINICAL REPORT

PATIENT INFORMATION

Patient Name:	PRIYA SHARMA	MRN:	APL-2026-048271
Date of Birth:	15-Mar-2002	UHID:	UH-0948-2026-BLR
Age / Gender:	24 Years / Female	Admission Date:	09-May-2026, 05:54 AM
Blood Group:	B+ (Positive)	Discharge Date:	11-May-2026, 05:54 AM
Aadhaar (last 4):	XXXX-XXXX-7842	Attending Doctor:	Dr. Meera Krishnan, MD (Pulm.)

CHIEF COMPLAINTS & PRESENTING ILLNESS

The patient presented to the Emergency Department with acute onset shortness of breath (dyspnea) and audible wheezing for the past 6 hours. She reported worsening symptoms triggered by exposure to construction dust near her workplace. She also complained of persistent chest tightness and mild palpitations. She has a known history of bronchial asthma since childhood with intermittent exacerbations. She denied fever, hemoptysis, or recent upper respiratory infection.

On further questioning, the patient also reported occasional headaches and dizziness over the past 2 months. She was found to have elevated blood pressure readings during triage (148/94 mmHg).

PAST MEDICAL HISTORY & COMORBIDITIES

> Mild Persistent Asthma (J45.30)	Diagnosed at age 8. Uses Salbutamol MDI PRN. Last exacerbation 4 months ago.
> Essential Hypertension, Stage 1 (I10)	Newly diagnosed during this admission. No prior antihypertensive therapy.
> Allergic Rhinitis (J30.1)	Seasonal. Managed with intranasal corticosteroids intermittently.
> Iron Deficiency Anemia - Resolved (D50.9)	Treated and resolved in 2024. Hb normalized.

VITAL SIGNS (Recorded at Admission)

Parameter	Value	Unit	Reference	Flag
Heart Rate (Pulse)	112	beats/min	60 - 100	H
Blood Pressure (Systolic)	148	mmHg	90 - 120	H
Blood Pressure (Diastolic)	94	mmHg	60 - 80	H
Oxygen Saturation (SpO2)	91	%	95 - 100	L
Respiratory Rate	26	breaths/min	12 - 20	H
Body Temperature	37.1	deg C	36.1 - 37.2	
BMI	22.4	kg/m2	18.5 - 24.9	

VITAL SIGNS (Recorded at Discharge)

Parameter	Value	Unit	Reference	Flag
Heart Rate (Pulse)	78	beats/min	60 - 100	
Blood Pressure (Systolic)	126	mmHg	90 - 120	H
Blood Pressure (Diastolic)	82	mmHg	60 - 80	H
Oxygen Saturation (SpO2)	97	%	95 - 100	
Respiratory Rate	16	breaths/min	12 - 20	
Body Temperature	36.8	deg C	36.1 - 37.2	

LABORATORY INVESTIGATIONS

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Test	Result	Unit	Reference	Flag
Hemoglobin	12.8	g/dL	12.0 - 15.5	
Total WBC Count	9,200	/cumm	4,000 - 11,000	
Platelet Count	2.4	Lakh/cumm	1.5 - 4.0	
ESR	18	mm/hr	0 - 20	
Serum IgE (Total)	485	IU/mL	< 100	H
Serum Creatinine	0.8	mg/dL	0.6 - 1.2	
Fasting Blood Glucose	92	mg/dL	70 - 100	
HbA1c	5.2	%	< 5.7	
Total Cholesterol	195	mg/dL	< 200	
Serum Sodium	140	mEq/L	136 - 145	
Serum Potassium	4.1	mEq/L	3.5 - 5.0	

PULMONARY FUNCTION TEST (SPIROMETRY)

Parameter	Pre-BD	Post-BD	Predicted
FEV1	2.1 L (72%)	2.6 L (89%)	2.92 L
FVC	3.0 L (85%)	3.2 L (91%)	3.52 L
FEV1/FVC Ratio	70%	81%	> 75%

Interpretation: Mild obstructive pattern with significant bronchodilator reversibility (> 12%). Consistent with asthma.

TREATMENT ADMINISTERED DURING HOSPITALIZATION

- > Nebulization with Salbutamol 2.5mg + Ipratropium 0.5mg Q6H for 48 hours
- > IV Hydrocortisone 100mg STAT, followed by oral Prednisolone 40mg OD x 5 days (tapering)
- > Oxygen supplementation via nasal cannula at 2L/min for 12 hours
- > Tab. Montelukast 10mg OD (added for long-term asthma control)
- > Tab. Amlodipine 5mg OD (initiated for newly diagnosed hypertension)
- > Fluticasone/Salmeterol MDI 250/50mcg 1 puff BD (step-up controller therapy)
- > IV Normal Saline 0.9% for hydration during first 24 hours

DISCHARGE PRESCRIPTION

Medication	Dosage	Frequency	Duration
Tab. Amlodipine	5 mg	Once daily (AM)	30 days
Tab. Montelukast	10 mg	Once daily (HS)	30 days
Tab. Prednisolone	20 mg	OD (tapering)	5 days
Fluticasone/Salmeterol MDI	250/50 mcg	1 puff BD	Long-term
Salbutamol MDI (Rescue)	100 mcg	2 puffs PRN	As needed
Tab. Cetirizine	10 mg	Once daily	14 days

FINAL DIAGNOSIS (ICD-10 CODED)

- | | | | |
|----|---|--------|--------|
| 1. | Acute Exacerbation of Mild Persistent Bronchial Asthma | J45.31 | Active |
| 2. | Essential (Primary) Hypertension, Stage 1 - Newly Diagnosed | I10 | Active |
| 3. | Allergic Rhinitis, Perennial | J30.1 | Active |

FOLLOW-UP & RECOMMENDATIONS

- > Follow-up with Pulmonology OPD in 2 weeks (Dr. Meera Krishnan)
- > BP monitoring at home twice daily. Maintain a log for review

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- > Follow-up with Internal Medicine in 4 weeks for BP reassessment
- > Avoid known allergens (dust, pollen, strong perfumes)
- > Carry Salbutamol rescue inhaler at all times
- > Repeat spirometry in 6 weeks
- > Diet: Low sodium (< 5g/day), adequate hydration
- > If SpO2 drops below 92% or severe breathlessness - visit ER immediately

Dr. Meera Krishnan

MD (Pulmonary Medicine)
Consultant Pulmonologist
KMC Reg: 78452-KA

Dr. Arjun Patel

MBBS, DNB (Internal Medicine)
Resident, Internal Medicine
KMC Reg: 94201-KA